Trastuzumab emtansine (T-DM1) · metastatic

T-DM1 3.6 mg/kg intravenous (IV) every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Source reconciled; independent clinical, pharmacy and patient review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

Trastuzumab emtansine (T-DM1)

New cough or breathlessness: seek prompt assessment.

3 Liver

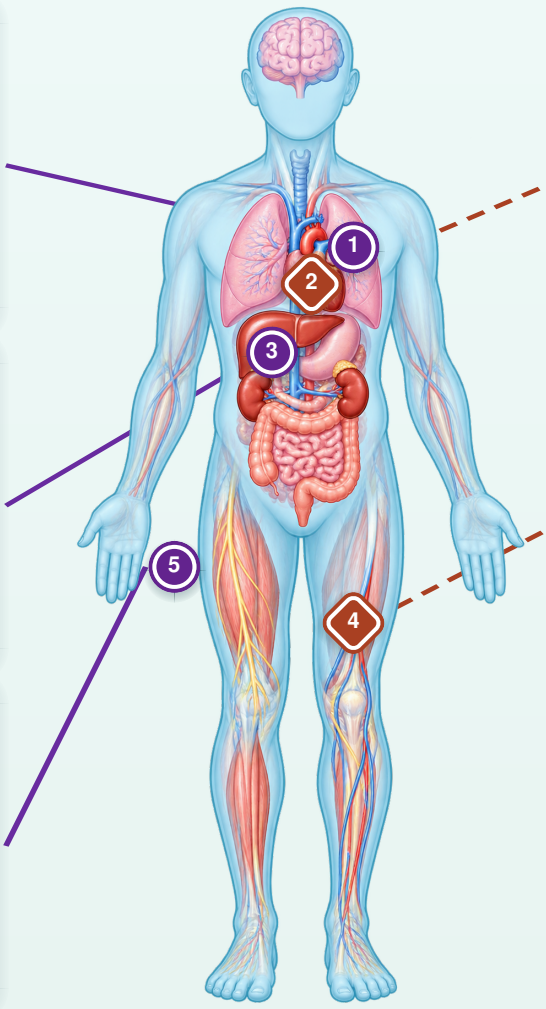
T-DM1

Keep liver blood tests. Report yellow eyes / skin, dark urine or significant abdominal symptoms.

5 Nerves

T-DM1

Report numbness or tingling, especially if it affects walking or hand use.



2 Heart

Trastuzumab emtansine (T-DM1)

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.

4 Platelets / bleeding

T-DM1

Report unexplained bruising or bleeding; heavy or persistent bleeding needs urgent help.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Trastuzumab emtansine (T-DM1)

- Low platelets and bleeding
- Liver injury, rarely serious
- Numbness or tingling
- Heart pumping-function reduction
- Rare serious lung inflammation
- Fatigue, nausea and infusion reactions

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Support for this medicine

T-DM1 has low emetogenic risk; review the low-risk antiemetic draft and individual contraindications.

Blood and heart appointments

full blood count (FBC), renal and liver profile before every cycle. Echo / multigated acquisition (heart pumping) scan (MUGA) every 12 weeks; around C5 for uninterrupted 21-day cycles.

Other treatments and bone health

Radiotherapy or local procedures are used only if indicated. Any endocrine or other systemic treatment needs a separate oncology plan. Bone-health checks depend on the individual risk and treatment context.

Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00206/

Sources and review status are available online. Follow your own prescription and hospital instructions.